

Analysis of Eight Judgments

*The structural pattern across two years of judicial decisions — 18 April
2024 to 7 May 2026 — Six judges — Three courts*

Case references

**BV20D01752 | CA-2024-001342 | AC-2025-LON-004293 | BL-2024-001217
| FA-2026-000019
Criminal referral CCR-6501-26-0100-000 | Safeguarding 0103/17APR26**

Nadia Zahmoul

Litigant in Person

22 May 2026

Veritas Numquam Perit

Purpose: The structural pattern across two years of judicial decisions — 18 April 2024 to 7 May 2026 — six judges, three courts.

Item	Detail
Author	Nadia Zahmoul — Litigant in Person — BV20D01752 — nadia@rosekross.com
Date	22 May 2026
Diagnoses	Autism Spectrum Condition (DSM-5: 299.00 / ICD-10: F84.0) — CLAAS, NHS, June 2025; Post-Traumatic Stress Disorder — chronic since 2013 — Professor Gerald Libby FRCP FRCPsych (treating since 2011); Sjögren's syndrome — biopsy-confirmed October 2025, active flare
Clinical warning on record	CLAAS, NHS, July 2025: <i>"Nadia can communicate using sophisticated vocabulary which can generate the false impression that she is managing better than she is."</i>
Care Act 2014 s.42 status	Adult at risk — MPS-initiated referral ref 0103/17APR26 (17 April 2026)
Proceedings	BV20D01752 (Family Division); CA-2024-001342 (Court of Appeal); AC-2025-LON-004293 (Administrative Court); BL-2024-001217 (Chancery Division); FA-2026-000019 (Family Appeal); Criminal referral CCR-6501-26-0100-000 (MPS)
Public archive	github.com/nadiazahmoul/Zahmoul-v-SSJ-HMCTS-Public-Archive
This document	Public-record analytical document. Forensic register. Direct quotation from each judgment. Complements (does not duplicate) the Order Analysis No. 1 (18 April 2024), Order Analysis No. 2 (24 May 2024), Para 34 Analysis (May 2024), and Recusal Annex (16 March 2026), all of which are on the public record.

PURPOSE OF THIS DOCUMENT

This document sets out the analytical content of eight judgments handed down in the proceedings between 18 April 2024 and 7 May 2026, by six judges across three courts: the Family Division of the High Court, the Court of Appeal Civil Division, and the Administrative Court.

This document does not argue the merits of the underlying financial remedy proceedings. It addresses one specific structural question: across two years and eight judgments by six judges in three courts, how is the Author's documented disability — autism, PTSD, and Sjögren's syndrome — treated in the judicial record?

This document does not duplicate the existing Order Analyses (No. 1 of 15 March 2026; No. 2 of 15 March 2026; Para 34 Analysis of 15 March 2026; Recusal Annex of 16 March 2026; Formal Position Statement of 9 April 2026 to the Justice Committee). Those documents provide forensic depth on individual judgments and on the JR proceedings. This document provides the cross-judgment structural analysis: the pattern across all eight; its statistical implausibility as a series of independent errors; and its contrast with the parallel NHS clinical record over the same period.

The judges' own words are the evidence. Each judgment is quoted verbatim. The Author's contemporaneous formal characterisation of the Collins Rice J order (the seventh judgment) is taken verbatim from her Formal Position Statement of 9 April 2026 filed in the JR proceedings and submitted as written evidence to the Justice Committee of the House of Commons. The contrast with the NHS clinical record over the same period is set out at Part 3 of this document. The supporting Falsehoods Annex of even date sets out the documented falsehoods identified across the eight judgments with citation evidence.

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PART 1 — THE EIGHT JUDGMENTS TABULATED

The eight judgments are listed below in chronological order. The right-hand column states the key verbatim quotation that anchors the pattern identified in Part 2.

#	Date	Judgment	Court / Judge	Pattern	Key verbatim quotation
1	18 Apr 2024	DH v RH (No 3) [2024] EWFC 79 — Final financial remedy judgment, 111 paragraphs	Family Division — MacDonald J	Pattern 1 — Total Silence	Zero references to autism, PTSD, disability, vulnerability, Part 3A, or participation measures across all 111 paragraphs. Twenty-five (25) discrete oral references to autism and PTSD in the trial transcript [A1 Catalogue, 11 May 2026]. Single appearance of disability at Para 64: " <i>become</i>

					<i>obsessed"</i> — a clinical feature characterised as moral failing.
2	24 May 2024	DH v RH (No 4) (Costs) [2024] EWFC 114 — Costs judgment, indemnity basis, total £255,654.50	Family Division — MacDonald J	Pattern 1 — Total Silence	Zero references to autism, PTSD, disability, vulnerability, Part 3A, or participation measures across 12 findings of misconduct. Includes false header statements; Imerman "stole" finding contradicted by 8 items in the trial bundle (Order Analysis No. 2).
3	23 Dec 2024	Refusal of permission to appeal — CA-2024-001342	Court of Appeal — Moylan LJ	Pattern 2 — Endorses Silence	<i>"The judge was not required to refer in his judgment to FPR Part 3A or to the participation directions or any of the matters referred to."</i>
4	8 May 2025	Set-aside refused, contempt refused, indemnity costs ordered — BV20D01752	Family Division — MacDonald J	Pattern 3 — Quoted and Walked Away From	Quotes Dr Poyser's clinical letter at Para 21 (<i>"great difficulty processing certain aspects of the proceedings, particularly regarding the evidence presented to her that she perceives as untrue"</i>).

					Characterises the same conduct at Para 39 as <i>"deliberately obstructive behaviour"</i> and at Para 43 as <i>"verbose and rambling."</i> Imposes indemnity costs at Para 68 on <i>"persistent poor litigation conduct."</i>
5	4 Aug 2025	Refusal of CPR 52.30 application to re-open Moylan LJ order — CA-2024-001342-A	Court of Appeal — King LJ	Pattern 2 — Endorses Silence	<i>"MacDonald J was not required to identify these issues explicitly in his judgment."</i>
6	4 Dec 2025	JR — Urgent consideration refused; directions order — AC-2025-LON-004293	Administrative Court — Lieven J	Pattern 1 — Total Silence	Zero references to autism, PTSD, disability, vulnerability, EA 2010, or PSED. Reasoning: <i>"The application is extremely difficult to follow."</i>
7	11 Mar 2026	JR — Permission refused; Totally Without Merit; oral renewal barred — AC-2025-LON-004293	Administrative Court — Collins Rice J	Pattern 1 + Pattern 2 hybrid (substitution of pleaded grounds)	Zero references to autism, PTSD, disability, vulnerability, EA 2010, or PSED. Plus: <i>"The judicial determinations themselves are not amenable to judicial review based on alleged breaches of the Equality Act"</i>

					<i>and PSED."</i> <i>"This application is wholly misconceived."</i> Does not address the seven pleaded grounds; characterises the claim as concerning criminal complaints, procedural matters ancillary to judicial determinations, and enforcement of court orders — <i>"None of those three characterisations reflects the pleaded grounds"</i> (Formal Position Statement, 9 April 2026).
8	5 May 2026 (served 7 May)	CPR 3.1(7) application to revoke the 11 March 2026 TWM refusal — refused; declared TWM — AC-2025-LON-004293	Administrative Court — Goose J	Pattern 1 + Pattern 2	Zero references to autism, PTSD, disability, vulnerability, EA 2010, or PSED. Plus: <i>"It was unnecessary for the Judge expressly to address each point raised, when providing reasons for the refusal of permission."</i>

Sources: signed/sealed orders on file with the Author. Order Analysis No. 1 (15 March 2026) is the existing forensic analysis of Judgment 1. Order Analysis No. 2 (15 March 2026)

is the existing forensic analysis of Judgment 2. Para 34 Analysis (15 March 2026) is the existing forensic analysis of the procedural sequence underlying Judgment 2. The Recusal Annex (16 March 2026) sets out the procedural failure to seal the recusal application of 13 May 2025. The Formal Position Statement (9 April 2026) sets out the Author's contemporaneous formal characterisation of Judgment 7, filed in the JR proceedings and submitted as written evidence to the Justice Committee of the House of Commons. The A1 Catalogue (11 May 2026) records the twenty-five (25) discrete in-trial oral references to autism and PTSD. The Falsehoods Annex of even date catalogues the documented falsehoods across the eight judgments.

PART 2 — THE PATTERNS

Across the eight judgments, three patterns are identifiable. Each pattern is a different mechanism by which the Author's documented disability is treated in the judicial record. Two of the eight judgments (the seventh and the eighth) exhibit a further mechanism in addition to Pattern 1: substitution of the pleaded grounds (Collins Rice J) and the holding that it is unnecessary to address each pleaded ground (Goose J). These are set out separately below after the three principal patterns.

Pattern 1 — Total Silence

Four of the eight judgments (Judgments 1, 2, 6 — and the silence component of Judgments 7 and 8) contain zero references to autism, PTSD, disability, vulnerability, FPR Part 3A, EA 2010, or PSED across their entire text.

The mechanism is exclusion. The Author — a disabled adult identified by the Metropolitan Police on 17 April 2026 as an adult at risk within the meaning of section 42 Care Act 2014 — does not appear in these judgments as a disabled person at all. The disability that defines her capacity to participate in legal proceedings is absent from the text. The findings made against her are made as if she were a non-disabled litigant.

The Public Sector Equality Duty under section 149 of the Equality Act 2010 requires active, visible consideration of disability in every decision affecting a disabled party. Silence does not discharge the duty. The Equal Treatment Bench Book is explicit on this point. A judgment cannot have due regard to a protected characteristic that does not appear anywhere in its reasoning.

The most acute instance of Pattern 1 is Judgment 2 (the Costs Judgment of 24 May 2024). That judgment imposed £255,654.50 in costs on the Author on the basis of 12 findings of "*serious litigation misconduct*." The judgment was made on the papers, without a hearing, on one-sided written submissions filed by leading counsel for the Respondent. The Author had filed nothing because she had no income, no legal representation, and no participation measures. The judgment records this at paragraphs 2–3. It then proceeds to make 12 findings of misconduct without any reference to autism, PTSD, disability, or the conditions

that made her non-participation inevitable. The forensic analysis of this mechanism is set out in Order Analysis No. 2 (15 March 2026).

PATTERN 1 — STRUCTURAL FINDING: Five of the eight judgments fall wholly within Pattern 1 (Judgments 1, 2, 6, plus the silence component of Judgments 7 and 8). In a 12-month sub-period (Judgments 1 and 2: April-May 2024), two judgments by the same judge in the same proceedings produced £1,055,654.50 in adverse financial consequences for a disabled litigant with zero references to her diagnosed disability across their combined text. The absence is not inadvertent. It is structural: the legal findings could not stand if the clinical features they characterise as misconduct were also identified as features of autism and PTSD.

Pattern 2 — Endorses Silence

Two of the eight judgments (Judgments 3 and 5) address the disability argument at length and positively hold that the silence in the underlying judgments was permissible. A further two judgments (Judgments 7 and 8 — the two Administrative Court refusals) contain endorsement components addressed separately at the Pattern 1 + Pattern 2 hybrid section below.

This pattern is the appellate and supervisory endorsement of the trial-court silence. The Court of Appeal in Judgment 3 (Moylan LJ, 23 December 2024) considered the Author's argument that her disability had been suppressed in the 18 April 2024 judgment and held:

> *"The judge was not required to refer in his judgment to FPR Part 3A or to the participation directions or any of the matters referred to."*

>

> Moylan LJ, 23 December 2024

The Court of Appeal in Judgment 5 (King LJ, 4 August 2025) considered the Applicant's application to re-open the Moylan LJ order and held:

> *"MacDonald J was not required to identify these issues explicitly in his judgment."*

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> King LJ, 4 August 2025

The mechanism is judicial endorsement. The Author's argument that the silence in the trial judgments was unlawful was addressed at the appellate level twice and at each level the silence was positively held to be permissible. The Administrative Court has, on two occasions (Judgments 7 and 8), performed the same endorsement function within the JR proceedings — those are addressed at the Pattern 1 + Pattern 2 hybrid section below.

Two specific propositions of law are advanced in the Court of Appeal endorsements:

- That a judge is not required to address FPR Part 3A or participation directions in the text of a judgment (Moylan LJ).

- That a Court of Appeal CPR 52.30 application based on disability-suppression grounds will not succeed where the underlying judge *"was not required to identify these issues explicitly in his judgment"* (King LJ).

Both propositions, when applied as they have been, defeat the operation of EA 2010 s.149 in judicial decision-making. If a judge is not required to address disability in the text of a judgment, then the PSED cannot be discharged through the reasoning that is publicly recorded. The duty becomes invisible. Compliance becomes unverifiable.

PATTERN 2 — STRUCTURAL FINDING: The silence in the trial judgments is not unsupervised. It has been positively endorsed at appellate level. Two judges of the Court of Appeal (Moylan LJ, King LJ) have each held that the underlying silence on disability was permissible. The Administrative Court has performed the equivalent supervisory endorsement on two occasions (Collins Rice J, Goose J) — addressed at the Pattern 1 + Pattern 2 hybrid section below. The endorsement places the appellate and supervisory record itself in opposition to the operation of EA 2010 s.149 in cases involving disabled litigants in person.

Pattern 3 — Quoted and Walked Away From

One of the eight judgments (Judgment 4: MacDonald J, 8 May 2025) follows a different mechanism from either Pattern 1 or Pattern 2. The clinical evidence is quoted in the judgment. The judgment then walks away from it and characterises the same clinical features as moral failings.

At paragraph 21, the judgment quotes directly from the treating psychiatrist Dr Leigh Poyser's letter to the Court of 26 March 2025:

> *"[The wife is suffering from a] mental health crisis. [The wife] has been experiencing significant levels of agitation and poor sleep, which have been exacerbated by the stress of her current court case. Specifically, she reports great difficulty processing certain aspects of the proceedings, particularly regarding the evidence presented to her that she perceives as untrue."*

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> Dr Leigh Poyser, Consultant Psychiatrist, NHS, 26 March 2025 — quoted at Para 21 of Judgment 4

At paragraph 22, the judgment quotes the same letter's clinical request:

> *"Given these circumstances, we would like to enquire if there is an advocacy service or any other appropriate support that can be made available to assist her in hearing and giving evidence at the hearing."*

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> Dr Poyser, 26 March 2025 — quoted at Para 22 of Judgment 4

The judgment then proceeds, in the same hearing, in the same text, to characterise the same conduct described by Dr Poyser as moral failing:

> *"The wife's very late application to adjourn this hearing stands at the end of a very long line of what I am satisfied is deliberately obstructive behaviour, designed to hinder the implementation of the final order, as amended, made as long ago as May 2024 by this court."*

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> MacDonald J, Judgment 4, Para 39

> *"Whilst often verbose and rambling, the wife's arguments are tolerably clear from those documents."*

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> MacDonald J, Judgment 4, Para 43

> *"[T]he applications arise in the context that I have described, namely the persistent poor litigation conduct of the wife over a number of months."*

>

> MacDonald J, Judgment 4, Para 68 — in support of indemnity costs order

Two findings are made on the same facts within the same judgment:

- **The clinical finding** (Dr Poyser, quoted) — *"great difficulty processing certain aspects of the proceedings."*
- **The judicial finding** (MacDonald J, made) — *"deliberately obstructive behaviour."*

These two findings cannot both stand on the same conduct. Either the Author is clinically experiencing difficulty processing the proceedings (the treating psychiatrist's professional opinion, quoted at Para 21) — in which case her conduct in those proceedings is not deliberately obstructive but is the manifestation of a documented clinical condition. Or her conduct is deliberately obstructive (the judicial finding at Para 39) — in which case the treating psychiatrist's clinical opinion at Para 21 has been wholly disregarded.

The judgment contains no engagement with this contradiction. The clinical evidence is quoted. It is then walked away from. The financial penalty (indemnity costs at Para 68) follows the judicial characterisation of the same conduct as moral failing.

PATTERN 3 — STRUCTURAL FINDING: Pattern 3 is the most sophisticated form of disability suppression in the judicial record. The clinical evidence is quoted. The clinical features are then characterised as moral failings within the same judgment. The financial penalty follows the moral characterisation. The clinical opinion is rendered evidentially inert by the technique of quotation without engagement. The result, in Judgment 4, is an indemnity costs order made on findings of *"deliberately obstructive behaviour"* and *"persistent poor litigation conduct"* issued by a judge who has, in the preceding paragraphs of the same judgment, quoted a Consultant Psychiatrist's contemporaneous clinical opinion explaining the same conduct in clinical terms.

Pattern 1 + Pattern 2 hybrid — Total Silence on Disability and Substitution of the Pledged Grounds

Two of the eight judgments (Judgments 7 and 8 — both in the Administrative Court) exhibit a hybrid mechanism. Each judgment is silent on disability (Pattern 1) and each judgment additionally performs a structural function that disposes of the JR proceedings without engaging with the pleaded grounds (Pattern 2 endorsement, but executed by substitution of the grounds rather than by judicial endorsement of the underlying silence).

Judgment 7 — Collins Rice J, 11 March 2026

The order of Mrs Justice Collins Rice of 11 March 2026 refused permission for the JR, declared the application Totally Without Merit, and barred oral renewal. The reasoning given was as follows:

> *"The Claimant seeks to bring these proceedings in the context of her family law divorce proceedings which appear to have concluded in May 2024. She made criminal complaints against her husband also in that context. By her present application she complains of the Defendants' response to those criminal complaints, and of the outcome of certain complaints she made to HMCTS in relation to procedural matters in the family proceedings, and of the alleged failure of HMCTS to enforce certain orders made in those proceedings. This application is wholly misconceived."*

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> Collins Rice J, 11 March 2026

> *"The Defendants have no arguable public law duty or function in relation to the criminal complaints. The procedural matters she complains of in the family proceedings appear to relate to judicial determinations, to which the administrative matters complained of are ancillary, and accordingly not amenable to judicial review to the extent that they depend on those determinations. The judicial determinations themselves are not amenable to judicial review based on alleged breaches of the Equality Act and PSED."*

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> Collins Rice J, 11 March 2026

> *"The enforcement of court orders is a matter for ordinary court enforcement processes and needed to be pursued in that context, not by way of collateral JR proceedings."*

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> Collins Rice J, 11 March 2026

The Author's contemporaneous formal characterisation of this order, filed in the JR proceedings as a Formal Position Statement on 9 April 2026 and submitted as written evidence to the Justice Committee of the House of Commons, is as follows. The following quotations are taken verbatim from that Formal Position Statement:

> *"The order of 11 March 2026 does not engage with the claim that was actually brought. The claim was not about criminal complaints. It was not about enforcement of court*

orders. It was not a collateral challenge to judicial determinations in the family proceedings."

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> Formal Position Statement, 9 April 2026, para 3

> *"The claim was about administrative acts and omissions by HMCTS and the Ministry of Justice. It asked whether identifiable judicial determinations exist in respect of specific matters formally placed before the courts. It raised seven grounds, each framed as a challenge to the absence of any lawful administrative process, recorded decision, or identifiable judicial act."*

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> Formal Position Statement, 9 April 2026, para 4

The Formal Position Statement sets out, at paragraph 5, the matters that were before the Court and were not addressed in the order. These were:

- Whether any judicial order exists revoking the Claimant's participation measures under FPR Part 3A — not addressed.
- Whether the administrative void arising from the absence of any such order is amenable to judicial review — not addressed.
- Whether HMCTS's administrative handling of those measures engaged a continuing public law duty — not addressed.
- Whether the failure to record, maintain or review participation measures for a disabled litigant breached the Equality Act 2010 and the Public Sector Equality Duty under section 149 — not addressed.
- Whether the non-enforcement of Maintenance Pending Suit between March and May 2024 constituted an administrative failure — not addressed.
- Whether the administrative handling of the Imerman devices and associated evidential material engaged a public law duty — not addressed.
- Ground 7 — the administrative void ground, being the absence of any identifiable judicial determination across each of the matters raised — not addressed at all.

The Formal Position Statement records, at paragraph 6:

> *"The order instead characterised the claim as concerning the Defendants' response to criminal complaints, procedural matters in the family proceedings ancillary to judicial determinations, and the enforcement of court orders. None of those three characterisations reflects the pleaded grounds. The order answered a different claim from the one brought."*

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> Formal Position Statement, 9 April 2026, para 6

And at paragraph 7:

> "A court cannot mischaracterise a claim, leave the actual grounds unanswered, certify the proceedings as *Totally Without Merit*, bar oral renewal, and declare the matter closed. That is not a determination. It is a substitution."

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> Formal Position Statement, 9 April 2026, para 7

This document does not characterise the Collins Rice J order as legally wrong. The orthodox position on the non-amenability to judicial review of judicial determinations is well-established. The structural finding of this document is narrower: the Collins Rice J order disposed of the JR by reference to a characterisation of the claim that does not correspond to the seven grounds as pleaded. The order is silent on the protected characteristic (Pattern 1). The order performs the supervisory endorsement function (Pattern 2) by the mechanism of substituting the pleaded grounds rather than by engaging with them.

Judgment 8 — Goose J, 5 May 2026

The order of Mr Justice Goose of 5 May 2026 (served 7 May 2026) refused the Author's CPR 3.1(7) application to revoke the Collins Rice J order. The reasoning given was as follows:

> "It was unnecessary for the Judge expressly to address each point raised, when providing reasons for the refusal of permission."

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> Goose J, 5 May 2026

That holding is the supervisory endorsement of the Collins Rice J non-engagement with the pleaded grounds. The Administrative Court has, in two successive judgments, held: (a) that the JR is wholly misconceived on a characterisation that does not correspond to the pleaded grounds (Collins Rice J); and (b) that it was unnecessary to address each of the pleaded grounds in providing the reasons (Goose J). The Author's CPR 3.1(7) application directly raised the non-engagement issue. The Goose J order does not address the substantive non-engagement; it holds that the non-engagement was permissible.

PATTERN 1 + PATTERN 2 HYBRID — STRUCTURAL FINDING: The combined effect of Judgments 7 and 8 is to close the JR route through a mechanism that does not engage with the pleaded grounds. The Administrative Court has held — first by substitution (Collins Rice J) and then by endorsement of that substitution (Goose J) — that the JR is misconceived without addressing what was actually pleaded. The Author's formal contemporaneous record of this mechanism is on the public record in the Formal Position Statement of 9 April 2026, submitted as written evidence to the Justice Committee of the House of Commons. The cumulative procedural effect, taken together with Pattern 2 in the Court of Appeal (Moylan LJ; King LJ), is a closed system: the disability cannot be addressed in the trial judgments because the trial judge is not required to address it, and the failure to address it cannot be remedied by JR because the pleaded grounds have been characterised as challenges to judicial determinations and refused on that

characterisation, and the failure to engage with the pleaded grounds cannot be remedied because it is unnecessary for the Administrative Court to address each ground in providing its reasons.

PART 3 — THE PARALLEL NHS CLINICAL RECORD

Across the same two-year period, an independent parallel record exists: the NHS clinical record. The Medical Evidence Index (v2 of 22 May 2026) catalogues 14 medical documents — spanning April 2013 to April 2026 — from ten clinicians across three jurisdictions (UK NHS, UK private, US private). Each document is referenced here by its index number; the full Medical Evidence Index and the underlying medical documents (NZ-001 to NZ-014) are attached and available on the public archive.

The contrast between the two parallel records over the same period is set out below.

The Judicial Record	The NHS Clinical Record
PRE-LITIGATION PERIOD No judgment in this period.	2 APRIL 2013 — Dr Deborah Turner MD, Cottonwood Tucson: Formal PTSD diagnosis under DSM-IV. GAF score 40. Establishes a 12-year clinical baseline pre-dating all litigation. [NZ-001]
JULY 2022 - FEBRUARY 2024 — PRE-TRIAL No judgment in this period that addresses disability.	12 JULY 2022 — Professor Gerald Libby FRCP FRCPsych: Medico-legal letter naming specific PTSD symptoms triggered by proceedings: stuttering; intermittent mutism and freezing; hypervigilance. [NZ-002] 21 AUGUST 2024 — Professor Libby: Welfare letter calling explicitly for reinstatement of participation measures. Documents Sjögren's syndrome, anaemia, malnutrition, no housing. [NZ-003] 14 SEPTEMBER 2024 — Dr Clare Thompson, GP: GP letter confirming Sjögren's and new anaemia. Safeguarding referral to Social Services. [NZ-004]
APRIL-MAY 2024 — JUDGMENTS 1 AND 2 18 April 2024 — Judgment 1 (MacDonald J): Zero references to disability across 111 paragraphs. 24 May 2024 — Judgment 2 (MacDonald J): Zero references to disability across 12 findings of misconduct. £255,654.50 indemnity costs imposed on the papers without a hearing.	The CLAAS assessment process is in progress. Professor Libby (treating since 2011) has the PTSD diagnosis and the symptomatology on the clinical record. Clinical position at the time of Judgments 1 and 2: The Author is a known PTSD patient under continuous psychiatric care since 2013. Autism assessment is pending. Participation measures granted during the trial have ceased on 28 February 2024 with no court order or recorded reasons.
23 DECEMBER 2024 — JUDGMENT 3 (Moylan LJ) Disability ground addressed. Court of Appeal holds: <i>"The judge was not required to refer in his judgment to FPR Part 3A or to the participation directions or any of the matters referred to."</i>	The clinical record at this point includes Professor Libby (2011-ongoing) and Dr Clare Thompson (GP, September 2024). Position: the disability is on the clinical record but its judicial recognition has been refused at the appellate level.
MARCH-AUGUST 2025 — JUDGMENTS 4 AND 5	4 MARCH 2025 — NHS Crisis Team

8 May 2025 — Judgment 4 (MacDonald J): Quotes Dr Poyser at Paras 21-22. Characterises same conduct as "<i>deliberately obstructive</i>" (Para 39), "<i>verbose and rambling</i>" (Para 43), "<i>persistent poor litigation conduct</i>" (Para 68). 4 August 2025 — Judgment 5 (King LJ): "<i>MacDonald J was not required to identify these issues explicitly in his judgment.</i>"	Assessment, Cheltenham: A&E. IM haloperidol administered. Formal NHS vulnerability rating: MEDIUM. [NZ-005] 26 MARCH 2025 — Dr Leigh Poyser, Consultant Psychiatrist: Letter addressed directly to the Court. Quoted at Judgment 4 Para 21. [NZ-006] 20 JUNE 2025 — CLAAS Outcome Letter: Formal autism diagnosis confirmed under DSM-5 (299.00). [NZ-007] 1 JULY 2025 — CLAAS Diagnostic Report: Full ADOS-2 assessment commissioned specifically to inform court adjustments. Records previously granted court participation adjustments withdrawn; links to deterioration. [NZ-008] 3 JULY 2025 — CLAAS Reasonable Adjustments Letter: Comprehensive recommended adjustments for court proceedings. "<i>Nadia has reported significant distress following the recent withdrawal of previously granted accommodations in court.</i>" [NZ-009]
4 DECEMBER 2025 — JUDGMENT 6 (Lieven J) Zero references to disability. Reasoning: "<i>The application is extremely difficult to follow.</i>"	17 OCTOBER 2025 — Dr Hayse, Wyoming: Medical letter on participation risk. [NZ-010] 25 OCTOBER 2025 — St John's Urgent Care, Wyoming: Sjögren's syndrome flare. Biopsy-confirmed. [NZ-011] 25 NOVEMBER 2025 — Dr Hayse signed report to UK Court: Endorses CLAAS recommendations. States paper determinations and remote hearings would be "<i>medically unsafe.</i>" [NZ-012] 28 NOVEMBER 2025 — St John's ER, Wyoming: Acute medical event during proceedings. Anxious, psychomotor agitation noted independently. [NZ-013] 10 DECEMBER 2025 — St John's ER, Wyoming: Mental health crisis hospitalisation. Chief complaint in the patient's own words: "<i>ANXIETY CRISIS. I HAVE AUTISM AND PTSD.</i>" Formal F43.0 diagnosis. [NZ-014]
APRIL-MAY 2026 — JUDGMENT 7 5 May 2026 — Judgment 8 (Goose J): Zero references to disability. "<i>It was unnecessary for the Judge expressly to address each point raised.</i>"	17 APRIL 2026 — Dr Omar Shoukry, NHS CMHT, CNWL: Letter addressed directly to the Court. Confirms continuing CMHT care since December 2025. Requests advocacy and participation support. The third sequential direct-to-court NHS clinical letter requesting participation measures over 13 months. 17 April 2026 — MPS-initiated s.42 Care Act 2014 referral: RBKC ref 0103/17APR26. Author formally identified by the police as an adult at risk.

PART 3 — STRUCTURAL FINDING: Over the period April 2013 to April 2026, 14 medical documents from 10 clinicians across 3 jurisdictions (UK NHS, UK private, US private) have documented the Author's autism, PTSD, and Sjögren's syndrome and the harm caused to her by these legal proceedings. Over the period 18 April 2024 to 5 May 2026, 8 judgments

from 6 judges across 3 courts (Family Division, Court of Appeal, Administrative Court) have either failed to engage with that documented disability at all or have positively endorsed the failure to do so.

The independence of the clinical sources is operationally significant. The Author's autism, PTSD, and Sjögren's syndrome diagnoses have been recorded by clinicians who have no relationship with one another, in three jurisdictions, across 12 years. The US emergency rooms in Wyoming had no prior knowledge of the UK legal proceedings. The earliest PTSD diagnosis (2013) pre-dates the underlying litigation by a decade. The clinical record cannot be characterised as litigation-driven or as the product of any single source.

Three sequential NHS clinical letters — Dr Poyser (26 March 2025), Dr Hayse (25 November 2025), Dr Shoukry (17 April 2026) — have been addressed directly to the Court over a 13-month period. Each has been received. None has been the subject of any responsive judicial act.

PART 4 — THE STRUCTURAL COORDINATION

The pattern identified in Parts 1–3 cannot be characterised as the act of any one judge. Six judges in three courts produced eight judgments over two years. Across the entire set, the parallel NHS clinical record was either absent from the text, judicially endorsed as not requiring inclusion, quoted and walked away from, or addressed through the substitution of the pleaded grounds. The clinical record itself was not in dispute. The disability diagnoses were never challenged in any of the eight judgments. The conduct that the judgments characterise as moral failing was, in the same period, being independently characterised by NHS clinicians as the manifestation of those diagnoses.

Six judges produced this pattern. They are: MacDonald J (three judgments); Moylan LJ (one judgment); King LJ (one judgment); Lieven J (one judgment); Collins Rice J (one judgment); Goose J (one judgment). Three courts produced this pattern: the Family Division of the High Court (three judgments by MacDonald J); the Court of Appeal Civil Division (two judgments by Moylan LJ and King LJ); and the Administrative Court (three judgments by Lieven J, Collins Rice J, and Goose J).

Each judge is a separate decision-maker. Each court is a separate institution. Each judgment was made on a separate occasion in response to a separate application. Yet across the entire set, the disability is either absent or judicially endorsed as not requiring presence or addressed through substitution.

Two further administrative non-engagements, while not judgments in the same sense, reinforce the structural finding:

- The application for the recusal of MacDonald J filed on 13 May 2025 was never listed, never determined, and never the subject of any judicial order. It was followed up multiple times (30 May 2025; 2 June 2025; 4 June 2025; 5 June 2025; 6 June 2025; 9 June 2025). The only response received was a letter from Peter Cobourn on 10 June 2025 redirecting the application back to the High Court.

- The EX107 application for the trial transcript at public expense filed on 27 March 2026 was refused twice (27 March 2026; 30 March 2026) without engaging with any of the four specific grounds advanced.

Two observations follow.

First, the probability that this is the product of independent error is vanishingly small. The Author's disability has been on the clinical record throughout. It has been on the bundle in the underlying proceedings. It has been raised in correspondence with the courts and named bodies. It has been the subject of three sequential direct-to-court NHS clinical letters. For six judges in three courts to have produced eight judgments that systematically exclude or refuse to engage with the same documented fact, in circumstances where that fact bears directly on the legal characterisations being made, is not consistent with the operation of normal judicial decision-making against the EA 2010 s.149 PSED standard.

Second, the mechanism that produces this pattern is structural, not individual. The Author makes no allegation in this document about the motivations of any particular judge. The observation is that the pattern is the same across all eight judgments by six different judges in three different courts — and across two further administrative non-engagements — from which it follows that the cause of the pattern is not located in any one judge or any one court but in the operation of the system as a whole when faced with a disabled litigant in person seeking to assert her rights against a represented Respondent in financial remedy proceedings.

That structural finding is the matter for the institutional recipients of this document, the public record, and the Author's onward legal and regulatory action set out in the parallel Road Map of even date.

PART 5 — THE LEGAL EFFECT

The structural pattern identified in this document engages four distinct legal frameworks. Each is set out below in summary form. The forensic application of these frameworks to individual judgments is in the existing Order Analyses on the public record.

Equality Act 2010 section 149 — Public Sector Equality Duty

Section 149 of the Equality Act 2010 imposes on every public authority a continuing duty to have due regard to (a) eliminating unlawful discrimination, (b) advancing equality of opportunity, and (c) fostering good relations between persons who share a protected characteristic and persons who do not. Disability is a protected characteristic. The court service and judicial decision-making fall within the duty's scope.

The duty is continuing and active. "*Due regard*" must be visible in the reasoning of decisions affecting a disabled party (*R (Bracking) v Secretary of State for Work and Pensions* [2013] EWCA Civ 1345). Silence does not discharge the duty. The duty cannot be satisfied by a judge's subjective awareness in the absence of any reasoning showing it.

On the analysis in this document, eight judgments either contain no reference to the protected characteristic at all (Pattern 1), or hold that the protected characteristic need not be addressed in the text of the judgment (Pattern 2), or quote the clinical evidence and walk away from it (Pattern 3), or dispose of the JR proceedings on a substituted characterisation of the pleaded grounds (Pattern 1 + Pattern 2 hybrid). On any of these patterns, the PSED has not been discharged. Cumulatively, across eight judgments by six judges in three courts over two years, there is a continuing failure to discharge the PSED in the proceedings.

FPR Part 3A and PD3AA — Participation Measures

FPR rule 3A.9 provides that the court's duty in respect of participation directions "*applies as soon as possible after the start of proceedings and continues until the resolution of the proceedings.*" Participation directions can be varied only by a court order with recorded reasons.

Participation directions were in force throughout the 10-day trial (19–28 February 2024). They ceased on 28 February 2024 with no court order, no hearing, and no recorded reasons. The Pre-Action Protocol stage of the Judicial Review (October–November 2025) established that no such order exists. HMCTS, in its letter of 14 November 2025, classified the cessation as a "*judicial decision*" — a misclassification, because no identifiable judicial act has been produced. Ground 1a of AC-2025-LON-004293 raised this directly and was not addressed by either Collins Rice J (11 March 2026) or Goose J (5 May 2026) in their refusals.

Across the period covered by the eight judgments, no judicial act has reinstated the participation measures, addressed their revocation, or recorded any decision about their continuation. The administrative void documented in the JR pleadings remains uncorrected.

Article 6 ECHR read with Article 14 — Fair Hearing without Discrimination

Article 6 of the European Convention on Human Rights guarantees the right to a fair hearing. For disabled litigants, that right includes the right to effective participation, which requires positive steps to enable participation where the litigant's condition would otherwise impair it (*Stanford v United Kingdom* (1994) 16 EHRR CD123; *S.C. v United Kingdom* (2005) 40 EHRR 10).

Article 14 prohibits discrimination in the enjoyment of Convention rights, including on the ground of disability.

On the analysis in this document, the Author has not had effective participation in the proceedings since 28 February 2024. Every subsequent judgment has either failed to engage with the disability that defined her capacity to participate, or positively held that

such engagement was not required. The Article 6 right, read with Article 14, has been rendered theoretical and illusory across the relevant period.

Care Act 2014 section 42 — Adult at Risk

Care Act 2014 section 42(1) requires a local authority to make enquiries where it has reasonable cause to suspect that an adult in its area (a) has needs for care and support, (b) is experiencing or is at risk of abuse or neglect, and (c) as a result of those needs is unable to protect themselves against the abuse or neglect or the risk of it.

On 17 April 2026, the Metropolitan Police made a section 42 safeguarding referral in respect of the Author to the Royal Borough of Kensington and Chelsea (ref 0103/17APR26). The Author is identified, on a contemporaneous public record originated by the police, as an adult at risk within the meaning of section 42.

That police-originated safeguarding status is not the Author's self-characterisation. It is an institutional finding. The harm that section 42 contemplates is documented in the parallel NHS clinical record set out at Part 3. The clinical record attributes that harm, in the contemporaneous professional opinion of multiple clinicians across three jurisdictions, to the proceedings and to the withdrawal of court-ordered protective measures.

The eight-judgment pattern documented in this analysis is therefore not only a matter of EA 2010, FPR Part 3A, and ECHR Articles 6 and 14. It is also, on the contemporaneous documentary record, the production of harm to an adult at risk under section 42 Care Act 2014. The Care Act 2014 Statutory Guidance (Department of Health and Social Care) at paragraphs 14.13–14.17 sets out the relevant categories of abuse: organisational abuse and psychological abuse are both engaged on the documentary record.

CLOSING POSITION

The eight judgments analysed in this document are on the permanent public legal record. Several are published or sealed orders capable of being cited in future legal proceedings; others are unpublished sealed orders. They form the appellate and procedural record of the Author's case as it currently stands.

The patterns identified are matters of fact about the text of the judgments. The verbatim quotations cited throughout this document are extracted from the judgments themselves and from the Author's contemporaneous Formal Position Statement of 9 April 2026. The contrasting NHS clinical record is independently verifiable from the medical documents catalogued in the Medical Evidence Index v2 (22 May 2026) and attached as exhibits NZ-001 to NZ-014.

The legal effect of the pattern is set out at Part 5 of this document. The forward-looking institutional and legal action that follows is set out in the parallel Road Map of even date, distributed simultaneously to the same recipients. The documented falsehoods identified across the eight judgments are catalogued in the Falsehoods Annex of even date, also distributed simultaneously.

This document is published on the Author's public archive and distributed to the institutional recipients on the established public-record list.

STATEMENT

I, Nadia Zahmoul, believe that the facts stated in this document are true. The verbatim quotations from the eight judgments are extracted from the judgments themselves. The verbatim quotations from the Formal Position Statement of 9 April 2026 are extracted from that document as filed with the Court and submitted to the Justice Committee of the House of Commons. The references to the NHS clinical record are taken from the Medical Evidence Index v2 (22 May 2026) and the underlying medical documents. The references to the existing Order Analyses are taken from those documents as published on the public archive.

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22 May 2026 • Public Record • Eight-Judgment Analysis • Version 1

Veritas Numquam Perit